



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : ثناء عبدالحكيم سيد حسن
المحافظة : القليوبية الحي / المدينة : شبرا الخيمة اول العنوان : ++++++
رقم البطاقة : 28104132501161 رقم التليفون : 01142482355 تاريخ تسجيل الإستمارة : 04-10-2026
العمر (تاريخ الميلاد) : 13-04-1981 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي الجسم [8530] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ----- التوقيع : -----
الاسم : د/ندى حسن الوظيفة : ----- التوقيع : -----
الاسم : د/نرمين رمضان الوظيفة : ----- التوقيع : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ثناء عبد الحكيم سيد حسن National ID: 28104132501161	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Whole-body PET/CT scan

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Patient Name	: الأستاذة / ثناء عبد الحكيم سيد	Request No.	: 132093
Patient No.	: 28104132501161	Request Date	: 12/12/2021
Path No.	: 3079/21	Verification Date	: 02/01/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Lt breast mass

Nature of the specimen:

Tru cut biopsy

Gross Examination:

Multiple gray white tissue cores up to 1.5 cm, totally submitted. Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type: IDC
- Grade II
- Angioinvasion: no
- Ductal carcinoma in situ: no
- Additional pathologic findings: Fat necrosis

Diagnosis:

Lt breast mass, tru cut biopsy:

Invasive duct carcinoma GII

Biomarkers:

ER: Strong nuclear reaction in 55% of tumor cells (Allred score 7/8)

PR: Strong nuclear reaction in 55% of tumor cells (Allred score 7/8)

Her-2: moderate complete membrane reaction in 10% of tumor cells (score 2+), for SISH

Ki 67: 15 % nuclear reaction

Prof. Gamal Abdelaty

Verified by :

Rana fathy

Printing Date : 03/01/2022 11:47:50

Page 1 of 2

Printed By : dr.Enas

Name: <u>ثناء عبد الحكيم سيد حسن</u>		
MR. : <u>6194</u>		
Detailed Medical Report		
		السيد
تجربة طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient underwent upfront left breast-conserving surgery with sentinel lymph node biopsy in January 2022. Final pathology was pT1N0, estrogen-receptor 7/8, progesterone-receptor 7/8, HER2-positive, Ki-67 15 %. Adjuvant therapy consisted of weekly Paclitaxel for 12 weeks combined with Herceptin, followed by continuation of subcutaneous Herceptin to complete a total of six months, and subsequent radiotherapy. The patient has been maintained on Tamoxifen since the completion of adjuvant treatment.		
An echocardiogram performed in February 2026 demonstrated normal left-ventricular function with an ejection fraction of 68 %. Imaging in April 2026 showed a bright, enlarged liver (hepatomegaly) and a mild left pleural effusion. A non-contrast chest CT was interpreted as normal. The patient continues to receive Tamoxifen as prescribed for the next three months.		
In April 2026, a follow-up scan confirmed the presence of a mild left pleural effusion and a bright liver consistent with hepatomegaly. A non-contrast chest CT was again reported as normal. The patient's Tamoxifen therapy was continued for an additional three months as ordered by pharmacy.		
During the multidisciplinary team meeting on 21 July 2026, the treatment plan was changed to oral capecitabine (Xeloda) combined with lapatinib (Tykerb). The decision to shift therapy was documented and the order was entered.		
At the multidisciplinary meeting in July 2026, the patient's prior treatment history was reviewed: upfront left-sided breast cancer surgery with sentinel node evaluation (pT1N0), adjuvant Paclitaxel for 12 weeks, Herceptin for nine months, radiotherapy, and ongoing Tamoxifen. Imaging in June 2026 revealed left pleural thickening measuring 12 mm, left pleural effusion, subcentimetric pulmonary nodules, and findings suggestive of metastatic disease. The MDT recommended a shift to Xeloda-Tykerb; however, this recommendation was rejected because the patient had not initiated intravenous Herceptin, and the case was referred back for further committee review.		
The most recent oncology note (31 August 2026) summarizes the patient's history of left-sided breast cancer (diagnosed 25 January 2022, pT1N0, ER 7/8, PR 7/8, HER2-positive, amplified) treated with 12 weeks of Paclitaxel, Herceptin (completed 2023), and Tamoxifen initiated June 2022. The patient now reports a chronic cough. CT imaging on 25 June 2026 shows left pleural nodular thickening, mild left pleural effusion, and subcentimetric pulmonary nodules, consistent with metastatic disease. A pleural true-cut biopsy performed in a private laboratory demonstrated necrotizing granulomatous pleuritis. The multidisciplinary team reviewed the pathology and imaging, and the patient was referred for reconsideration of Taxotere-Herceptin therapy pending slide revision and further imaging. The current plan is to reassess treatment options based on the revised pathology and imaging findings.		
تحريرا في ,		
التاريخ :	20-09-2026	الوقت :
الطبيب المعالج :	علياء احمد أبو النصر شهاب الدين	
يعتمد ,		

Patient Name : الأستاذ / ثناء عبد الحكيم سيد	Request No. : 13000912
Patient No. : 28104132501161	Request Date : 25/01/2022
Path No. : 244/22	Verification Date : 07/02/2022
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 42 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass

Nature of the specimen: Left WLE + Left SNLs + Wire guided specimen for intramammary LN

Gross Examination:

- Procedure: WLE measured 12x8x2cm, skin ellipse measured 10x4cm + Wire guided specimen for intramammary LN measured 7x4x1cm
- Localization: Left
- Tumor extent:
 - o Number: One ill defined greyish white mass
 - o Size: 2x1.5x0.5 cm
 - o Skin involvement: No
- Distance from margins: Least was the superficial margin (0.4cm)
- Lymph nodes number and size: SNL; 5 nodules up to 1x0.5cm + Intramammary nodule measured 0.5x0.5 cm

Microscopic examination:

- Histological type: Invasive Duct Carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (2+2+2)
- Angioinvasion: No
- o Skin involvement: Free
- Lymph node involvement: Free SNLs (0/5) + Free intramammary LN (0/1)
- Resection Margins: Free
- Additional Pathological findings: Positive perineural invasion

Diagnosis:

- Left breast cancer; WLE + SNLs + Wire guided specimen for intramammary LN:
- Invasive duct carcinoma GII

Verified by :

Patient Name	: الأستاذ / شاد عبد الحكيم سيد	Request No.	: 13000912
Patient No.	: 28104132501161	Request Date	: 25/01/2022
Path No.	: 244/22	Verification Date	: 07/02/2022
Sex	: Female	Financial Category	: أورام داء المصالح - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Positive perineural invasion
- Free margins and skin
- Total nodal status : 0/7
- pT1 pN0 pMx

Prof. Aml Hareedy
Dr. Menna Atef

Dr. Walaa Ghonaim.

Verified by :

Printing Date : 10/02/2022 13:02:56

Page 2 of 2

Printing By : marwa mohamed you

19402 25/12/2021

Name : Thanaa Abdulhakim Said Hassan Date : 24-08-2026 08:42
Branch : Dokki Medical No. : 446386
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعاقر مستشفى

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 45-year-old female with left breast carcinoma, status post partial mastectomy, chemotherapy, and radiotherapy. Presents with lower back pain. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Symmetrical homogeneous tracer accumulation throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild diffuse periarticular tracer concentration at knees and ankles, consistent with degenerative changes.
- Renal excretion is prompt. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- follow up as clinically indicated would be kindly advised.

Dr. Alaa Nagi MD Nuclear Medicine

Dr. Alaa Nagy



معامل الخلية تحليل الأنسجة والأورام

Patient Name	ثناء عبدالحكيم سيد حسن	Code Number	YK 26019551
Sex/Age	Female 45 Y 0 M 0 D	Patient ID	96942
Registered Date	25/6/2026 11:13:09 PM	Referred By Dr.	--
Validation Date	02/7/2026 03:26:27 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	02/7/2026 09:25:09 PM		

Clinical Diagnosis: Left Pleural Thickening
Nature of the specimen: U/S Guided TCNB

Pathology Report

Gross Picture

Two grey, white tissue cores with multiple fragments measured 5.3 cm in total length, totally submitted.

Microscopic Picture

Sections examined revealed wide areas of necrosis, surrounded by bands of histiocytes with multinucleated giant cells and mononuclear cells. No dysplasia or malignancy.

Diagnosis

Left Pleural Thickening, U/S Guided TCNB:
Necrotizing Granulomatous Pleuritis.
For Clinical, Radiologic And Microbiologic Correlation.

P.O

Number of slides enclosed: 1

Diagnosed by

Consultant Pathologist
Prof. Dr. Dalia Abouelfadl, MD



Scan Me



Name : Thanaa Abdulhakim Said Hassan Date : 24-08-2026 08:42
Branch : Dokki Medical No. : 446386
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعاقر مستشفيات

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 45-year-old female with left breast carcinoma, status post partial mastectomy, chemotherapy, and radiotherapy. Presents with lower back pain. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Symmetrical homogeneous tracer accumulation throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild diffuse periarticular tracer concentration at knees and ankles, consistent with degenerative changes.
- Renal excretion is prompt. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- follow up as clinically indicated would be kindly advised.

Dr. Alaa Nagi MD Nuclear Medicine

Dr. Alaa Nagy



معامل الخلية لتحليل الأنسجة والأورام

Patient Name	ثناء عبدالحكيم سيد حسن	Code Number	YK 26019551
Sex/Age	Female 45 Y 0 M 0 D	Patient ID	96942
Registered Date	25/6/2026 11:13:09 PM	Referred By Dr.	--
Validation Date	02/7/2026 03:26:27 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	02/7/2026 09:25:09 PM		

Clinical Diagnosis: Left Pleural Thickening
Nature of the specimen: U/S Guided TCNB

Pathology Report

Gross Picture

Two grey, white tissue cores with multiple fragments measured 5.3 cm in total length, totally submitted.

Microscopic Picture

Sections examined revealed wide areas of necrosis, surrounded by bands of histiocytes with multinucleated giant cells and mononuclear cells. No dysplasia or malignancy.

Diagnosis

Left Pleural Thickening, U/S Guided TCNB:
Necrotizing Granulomatous Pleuritis.
For Clinical, Radiologic And Microbiologic Correlation.

P.O

Number of slides enclosed: 1

Diagnosed by

Consultant Pathologist
Prof. Dr. Dalia Abouelfadl, MD



Scan Me



Name : Thanaa Abdulhakim Said Hassan Date : 24-08-2026 08:42
Branch : Dokki Medical No. : 446386
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعاقر مستشفيات

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 45-year-old female with left breast carcinoma, status post partial mastectomy, chemotherapy, and radiotherapy. Presents with lower back pain. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Symmetrical homogeneous tracer accumulation throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild diffuse periarticular tracer concentration at knees and ankles, consistent with degenerative changes.
- Renal excretion is prompt. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- follow up as clinically indicated would be kindly advised.

Dr. Alaa Nagi MD Nuclear Medicine

Dr. Alaa Nagy



معامل الخلية تحليل الأنسجة والأورام

Patient Name	ثناء عبدالحكيم سيد حسن	Code Number	YK 26019551
Sex/Age	Female 45 Y 0 M 0 D	Patient ID	96942
Registered Date	25/6/2026 11:13:09 PM	Referred By Dr.	--
Validation Date	02/7/2026 03:26:27 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	02/7/2026 09:25:09 PM		

Clinical Diagnosis: Left Pleural Thickening
Nature of the specimen: U/S Guided TCNB

Pathology Report

Gross Picture

Two grey, white tissue cores with multiple fragments measured 5.3 cm in total length, totally submitted.

Microscopic Picture

Sections examined revealed wide areas of necrosis, surrounded by bands of histiocytes with multinucleated giant cells and mononuclear cells. No dysplasia or malignancy.

Diagnosis

Left Pleural Thickening, U/S Guided TCNB:
Necrotizing Granulomatous Pleuritis.
For Clinical, Radiologic And Microbiologic Correlation.

P.O

Number of slides enclosed: 1

Diagnosed by

Consultant Pathologist
Prof. Dr. Dalia Abouelfadl, MD



Scan Me



Name : Thanaa Abdulhakim Said Hassan Date : 24-08-2026 08:42
Branch : Dokki Medical No. : 446386
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعاقر مستشفيات

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 45-year-old female with left breast carcinoma, status post partial mastectomy, chemotherapy, and radiotherapy. Presents with lower back pain. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Symmetrical homogeneous tracer accumulation throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild diffuse periarticular tracer concentration at knees and ankles, consistent with degenerative changes.
- Renal excretion is prompt. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- follow up as clinically indicated would be kindly advised.

Dr. Alaa Nagi MD Nuclear Medicine

Dr. Alaa Nagy



معامل الخلية تحليل الأنسجة والأورام

Patient Name	ثناء عبدالحكيم سيد حسن	Code Number	YK 26019551
Sex/Age	Female 45 Y 0 M 0 D	Patient ID	96942
Registered Date	25/6/2026 11:13:09 PM	Referred By Dr.	--
Validation Date	02/7/2026 03:26:27 PM	Branch	الفرع الرئيسي الدقي
Reporting Date	02/7/2026 09:25:09 PM		

Clinical Diagnosis: Left Pleural Thickening
Nature of the specimen: U/S Guided TCNB

Pathology Report

Gross Picture

Two grey, white tissue cores with multiple fragments measured 5.3 cm in total length, totally submitted.

Microscopic Picture

Sections examined revealed wide areas of necrosis, surrounded by bands of histiocytes with multinucleated giant cells and mononuclear cells. No dysplasia or malignancy.

Diagnosis

Left Pleural Thickening, U/S Guided TCNB:
Necrotizing Granulomatous Pleuritis.
For Clinical, Radiologic And Microbiologic Correlation.

P.O

Number of slides enclosed: 1

Diagnosed by

Consultant Pathologist
Prof. Dr. Dalia Abouelfadl, MD



Scan Me

